

Atracurium: Drug information - UpToDate

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Atracurium: Drug information

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For additional information see "Atracurium: Patient drug information" and "Atracurium: Pediatric drug information"

For abbreviations, symbols, and age group definitions show table

Pharmacologic Category

- Neuromuscular Blocker Agent, Nondepolarizing

Dosing: Adult

Note: Doses must be individualized due to interpatient variability, patient specific factors, and/or comorbidities (eg, severe trauma, burn injury) (Ref).

Endotracheal intubation

Endotracheal intubation: Note: Ensure adequate pain control and sedation prior to and during administration of neuromuscular blockade.

IV: 0.5 to 0.6 **mg**/kg once (Ref).

Mechanically ventilated patients in the ICU, neuromuscular blockade

Mechanically ventilated patients in the ICU, neuromuscular blockade:

Note: Ensure adequate pain control and sedation prior to and during administration of neuromuscular blockade. May use to facilitate mechanical ventilation (eg, refractory moderate to severe acute respiratory distress syndrome; refractory, life-threatening status asthmaticus; or shivering from therapeutic hypothermia) (Ref). Refer to institutional protocols. Titration specifics may vary; a titration example is provided below.

Continuous infusion : IV: 0.4 to 0.6 **mg**/kg initial loading dose, followed by continuous infusion of 4 to 12 **mcg**/kg/**minute**; titrate every ~5 to 10 minutes based on clinical response and neuromuscular monitoring (eg, train-of-four); usual dosage range: 2 to 20 **mcg**/kg/**minute** (Ref).

Intermittent dose : IV: 0.4 to 0.6 **mg**/kg, followed by 0.08 to 0.1 **mg**/kg every ~15 to 25 minutes based on clinical response and neuromuscular monitoring (eg, train-of-four) (Ref).

Neuromuscular blockade during general anesthesia

Neuromuscular blockade during general anesthesia (adjunctive therapy):

Maintenance dosing: Note: Inhaled anesthetic agents (eg, desflurane, isoflurane, sevoflurane) prolong the duration of action of atracurium and may potentiate the neuromuscular blockade.

Continuous infusion : IV: 4 to 12 **mcg/kg/minute** as an initial continuous infusion; titrate as needed based on clinical response and neuromuscular monitoring (eg, train-of-four); usual dosage range: 2 to 15 **mcg/kg/minute** (Ref).

Intermittent dose : IV: 0.08 to 0.1 **mg/kg** every ~15 to 25-minute intervals as needed based on clinical response and neuromuscular monitoring (eg, train-of-four) (Ref).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Adult

No dosage adjustment necessary.

Dosing: Liver Impairment: Adult

No dosage adjustment necessary.

Dosing: Obesity: Adult

The recommendations for dosing in patients with obesity are based upon the best available evidence and clinical expertise. Senior Editorial Team: Jeffrey F. Barletta, PharmD, FCCM; Manjunath P. Pai, PharmD, FCP; Jason A. Roberts, PhD, BPharm (Hons), B App Sc, FSHP, FISAC.

Class 1, 2, or 3 obesity (BMI ≥ 30 kg/m²):

IV: Use **ideal body weight** for weight-based dosing calculations, then titrate to clinical effect (Ref). When underdosing is a clinical concern, consider using **adjusted body weight** for weight-based dosing calculations, then titrate to clinical effect (Ref). Clinicians should **not** change dosing weight from one weight metric to another during therapy (ie, actual body weight to/from either adjusted body weight or ideal body weight) (Ref). Refer to adult dosing for indication-specific doses.

Rationale for recommendations: In data from both surgical populations and critically ill patients with acute respiratory distress syndrome, dosing of atracurium via ideal body weight has been associated with a predictable profile of muscle relaxation and clinical response as well as decreased overall drug exposure when compared to dosing based on total body weight (Ref).

Dosing: Older Adult

Refer to adult dosing.

Dosing: Pediatric

(For additional information see "Atracurium: Pediatric drug information")

Note: Ensure adequate pain control and sedation prior to and during administration of neuromuscular blockade. Dose to effect; doses must be individualized due to interpatient variability. Maintenance doses in infants and children may need to be administered with slightly greater frequency compared to adults. Dosing in obese patients should be calculated using ideal body weight (Ref):

Neuromuscular blockade

Neuromuscular blockade:

ICU paralysis (eg, facilitate mechanical ventilation) (Ref):

Infants, Children, and Adolescents:

Initial bolus: IV: 0.3 to 0.6 mg/kg/dose; repeat additional doses as needed to maintain desired neuromuscular blockade or begin continuous infusion.

Continuous IV infusion: Initial: 5 to 12 **mcg/kg/minute** (0.3 to 0.7 mg/kg/hour); range: 5 to 40 **mcg/kg/minute** (0.3 to 2.4 mg/kg/hour).

Adjunct to surgical anesthesia:

Bolus doses:

Infants and Children <2 years: Initial: IV: 0.3 to 0.4 mg/kg once, followed by additional doses as needed to maintain neuromuscular blockade.

Children ≥2 years and Adolescents: IV: 0.4 to 0.5 mg/kg once as initial dose, then administer 0.08 to 0.1 mg/kg/dose 20 to 45 minutes after the initial dose to maintain neuromuscular blockade; repeat dose every 15 to 25 minutes as needed. **Note:** Initial dose should be reduced to 0.3 to 0.4 mg/kg in patients with significant cardiovascular disease or with history of increased risk of histamine release (eg, asthma, severe anaphylactoid reaction).

Continuous IV infusion in operating room during extended surgical procedures:

Infants and Children <2 years: Continuous IV infusion: Initial: 6 to 14 **mcg**/kg/minute (0.4 to 0.8 mg/kg/**hour**) initiated at the first signs of recovery from initial bolus; titrate until desired neuromuscular blockade is achieved (Ref).

Children ≥2 years and Adolescents: Continuous IV infusion: Initial: 9 to 10 **mcg**/kg/minute (0.5 to 0.6 mg/kg/**hour**), initiate infusion at initial signs of recovery from bolus dose, titrate until desired neuromuscular blockade is achieved; block is usually maintained by a rate of 5 to 9 **mcg**/kg/minute (0.3 to 0.5 mg/kg/**hour**); range: 2 to 15 **mcg**/kg/minute (0.1 to 0.9 mg/kg/**hour**).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Pediatric

Infants, Children, and Adolescents: No dosage adjustment necessary.

Dosing: Liver Impairment: Pediatric

Infants, Children, and Adolescents: No dosage adjustment necessary.

Adverse Reactions

The following adverse drug reactions and incidences are derived from product labeling unless otherwise specified. Frequency not defined. Adverse reactions are mild, rare, and generally suggestive of histamine release.

1% to 10%: Cardiovascular: Flushing

<1%, postmarketing, and/or case reports: Bradycardia, bronchospasm, dyspnea, erythema, hypersensitivity reaction, hypotension, increased bronchial secretions, injection site reaction, laryngospasm, pruritus, seizure, tachycardia, urticaria, wheezing

Contraindications

Hypersensitivity to atracurium or any component of the formulation; known hypersensitivity to benzyl alcohol (multiple dose vials)

Warnings/Precautions

Concerns related to adverse effects:

- Anaphylaxis: Severe anaphylactic reactions have been reported with atracurium use; some life-threatening and fatal. Appropriate emergency treatment (including epinephrine 1 mg/mL) should be immediately available during use. Use caution in patients with previous anaphylactic reactions to other neuromuscular blocking agents.
- Bradycardia: May be more common with atracurium than with other neuromuscular-blocking agents since it has no clinically significant effects on heart rate to counteract the bradycardia produced by anesthetics.
- Prolonged paralysis: Some patients may experience prolonged recovery of neuromuscular function after administration (especially after prolonged use). Patients should be adequately recovered prior to extubation. Other factors associated with prolonged recovery should be considered (eg, corticosteroid use, patient condition).

Disease-related concerns:

- Burn injury: Resistance may occur in burn patients ($\geq 20\%$ of total body surface area), usually several days after the injury, and may persist for several months after wound healing (Han 2009).
- Conditions that may *antagonize* neuromuscular blockade (decreased paralysis): Respiratory alkalosis, hypercalcemia, demyelinating lesions, peripheral neuropathies, denervation, and muscle trauma may result in antagonism of neuromuscular blockade (ACCM/SCCM/ASHP [Murray 2002]; Greenberg 2013; Gropper 2019; Naguib 2002).
- Conditions that may *potentiate* neuromuscular blockade (increased paralysis): Electrolyte abnormalities (eg, severe hypocalcemia, severe hypokalemia, hypermagnesemia), neuromuscular diseases, metabolic acidosis, respiratory acidosis, Eaton-Lambert syndrome and myasthenia gravis may result in potentiation of neuromuscular blockade (Greenberg 2013; Gropper 2019; Naguib 2002).

Special populations:

- Older adults: Use with caution in older adults; effects and duration are more variable.
- Immobilized patients: Resistance may occur in patients who are immobilized.

Dosage form specific issues:

- Benzyl alcohol and derivatives: Some dosage forms may contain benzyl alcohol; large amounts of benzyl alcohol (≥ 99 mg/kg/day) have been associated with a potentially fatal toxicity (“gasping syndrome”) in neonates; the “gasping syndrome” consists of metabolic acidosis, respiratory distress, gasping respirations, CNS dysfunction (including convulsions, intracranial hemorrhage), hypotension and cardiovascular collapse (AAP [“Inactive” 1997]; CDC 1982); some data suggests that benzoate displaces bilirubin from protein binding sites (Ahlfors 2001); avoid or use dosage forms containing benzyl alcohol with caution in neonates. See manufacturer’s labeling.

Other warnings/precautions:

- Appropriate use: Maintenance of an adequate airway and respiratory support is critical. Resistance may develop with chronic treatment. All patients should receive eye care including liberal use of lubricating drops, gel, or ointment, and eyelids should remain closed during continuous neuromuscular blockade to protect against damage to the cornea (ulceration and drying).
- Experienced personnel: Should be administered by adequately trained individuals familiar with its use.
- Histamine release: Reduce initial dosage and inject slowly (over 1 to 2 minutes) in patients in whom substantial histamine release would be potentially hazardous (eg, patients with clinically-important cardiovascular disease).
- Risk of medication errors: Accidental administration may be fatal. Confirm proper selection of intended product, store vial so the cap and ferrule are intact and the possibility of selecting the wrong product is minimized, and ensure that the intended dose is clearly labeled and communicated, when applicable.

Dosage Forms: US

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Solution, Intravenous, as besylate:

Generic: 100 mg/10 mL (10 mL)

Solution, Intravenous, as besylate [preservative free]:

Generic: 50 mg/5 mL (5 mL)

Generic Equivalent Available: US

Yes

Pricing: US

Solution (Atracurium Besylate Intravenous)

50 mg/5 mL (per mL): \$1.14 - \$1.90

100 mg/10 mL (per mL): \$1.13 - \$1.90

Disclaimer: A representative AWP (Average Wholesale Price) price or price range is provided as reference price only. A range is provided when more than one manufacturer's AWP price is available and uses the low and high price reported by the manufacturers to determine the range. The pricing data should be used for benchmarking purposes only, and as such should not be used alone to set or adjudicate any prices for reimbursement or purchasing functions or considered to be an exact price for a single product and/or manufacturer. Medi-Span expressly disclaims all warranties of any kind or nature, whether express or implied, and assumes no liability with respect to accuracy of price or price range data published in its solutions. In no event shall Medi-Span be liable for special, indirect, incidental, or consequential damages arising from use of price or price range data. Pricing data is updated monthly.

Administration: Adult

IV: Administer undiluted as a bolus injection; do **not** administer IM (excessive tissue irritation). Continuous infusion administration requires the use of an infusion pump. Do not administer with alkaline solutions in the same syringe or simultaneously in the same IV line. Use infusion solutions within 24 hours of preparation.

Preparation for Administration: Adult

May prepare an infusion solution (final concentrations: 0.2 mg/mL, 0.5 mg/mL, 1 mg/mL, or 5 mg/mL) by admixing with an appropriate diluent (eg, NS, D5W, D5NS) (Ref). Do not mix with alkaline solutions.

Usual Infusion Concentrations: Adult

IV infusion: 20 mg in 100 mL (concentration: 0.2 mg/mL) **or** 50 mg in 100 mL (concentration: 0.5 mg/mL) **or** 250 mg in 250 mL (concentration: 1 mg/mL) **or** 500 mg in 100 mL (concentration: 5 mg/mL) of D5W, D5NS, or NS.

Administration: Pediatric

Parenteral: For IV use only; do not administer IM due to tissue irritation.

IV bolus: Administer undiluted as IV bolus injection.

Continuous IV infusions: Further dilute in a compatible fluid and administer via an infusion pump.

Preparation for Administration: Pediatric

Parenteral: Continuous IV infusion: Dilute in D5W, NS, or D5NS to a final concentration of 0.2 mg/mL or 0.5 mg/mL. Atracurium should not be mixed with alkaline solutions.

Storage/Stability

Store intact vials at 2°C to 8°C (36°F to 46°F). Do not freeze. Upon removal from refrigeration to room temperature storage conditions (25°C/77°F), use within 14 days even if re-refrigerated. Dilutions of 0.2 mg/mL or 0.5 mg/mL in 0.9% sodium chloride, dextrose 5% in water, or 5% dextrose in sodium chloride 0.9% are stable for up to 24 hours at room temperature or under refrigeration.

According to the 2020 to 2021 *ISMP Targeted Medication Safety Best Practices for Hospitals*, neuromuscular blockers should be segregated, sequestered, and differentiated from all other medication wherever they are stored.

Use: Labeled Indications

Endotracheal intubation: To facilitate endotracheal intubation.

Mechanically ventilated patients in the ICU, neuromuscular blockade: Provide skeletal muscle relaxation during mechanical ventilation in ICU patients.

Neuromuscular blockade during general anesthesia: As an adjunct to general anesthesia to provide skeletal muscle relaxation during surgery.

Note: Neuromuscular blockade does not provide pain control, sedation, or amnestic effects. Ensure adequate pain control and sedation prior to and during administration of neuromuscular blockade (Ref).

Medication Safety Issues

High alert medication:

The Institute for Safe Medication Practices (ISMP) includes this medication among its list of drug classes (neuromuscular blocker agent) which have a heightened risk of causing significant patient harm when used in error (ISMP List of High-Alert Medications in Acute Care Settings ©ISMP 2024).

Other safety concerns:

According to the 2020 to 2021 *ISMP Targeted Medication Safety Best Practices for Hospitals*, neuromuscular blockers should be segregated, sequestered, and differentiated from all other medication wherever they are stored. This includes:

Only storing in places within the hospital that they are routinely used.

Placing in sealed boxes or in rapid sequence intubation kits (preferred).

Limiting availability in automated dispensing cabinets to perioperative, labor and delivery, critical care, and emergency departments only.

Placing in separate lidded containers within the pharmacy refrigerator or other isolated pharmacy storage area.

Affixing an auxiliary label to clearly communicate respiratory paralysis will occur and ventilation required on all storage bins and/or automated dispensing pockets/drawers (exception anesthesia-prepared syringes) stating one of the following:

Warning: Causes Respiratory Arrest - Patient Must Be Ventilated

Warning: Paralyzing Agent - Causes Respiratory Arrest

Warning: Causes Respiratory Paralysis - Patient Must Be Ventilated

Metabolism/Transport Effects

None known.

Drug Interactions

(For additional information: Launch drug interactions program)

Note: Interacting drugs may **not be individually listed below** if they are part of a group interaction (eg, individual drugs within "CYP3A4 Inducers [Strong]" are NOT listed). For a complete list of drug interactions by individual drug name and detailed management recommendations, use the drug interactions program by clicking on the "Launch drug interactions program" link above.

Acetylcholinesterase Inhibitors: May decrease neuromuscular-blocking effects of Neuromuscular-Blocking Agents (Nondepolarizing). *Risk C: Monitor*

Aminoglycosides: May increase therapeutic effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Bacitracin (Systemic): May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Botulinum Toxin-Containing Products: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Bromperidol: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Calcium Channel Blockers: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents (Nondepolarizing). *Risk C: Monitor*

Capreomycin: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Carbamazepine: May decrease serum concentrations of Neuromuscular-Blocking Agents (Nondepolarizing). *Risk C: Monitor*

Cardiac Glycosides: Neuromuscular-Blocking Agents may increase arrhythmogenic effects of Cardiac Glycosides. *Risk C: Monitor*

Clindamycin (Topical): May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Colistimethate: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. Management: If possible, avoid concomitant use of these products. Monitor for deeper, prolonged neuromuscular-blocking effects (respiratory paralysis) in patients receiving concomitant neuromuscular-blocking agents and colistimethate. *Risk D: Consider Therapy Modification*

Corticosteroids (Systemic): Neuromuscular-Blocking Agents (Nondepolarizing) may increase adverse neuromuscular effects of Corticosteroids (Systemic). Increased muscle weakness, possibly progressing to polyneuropathies and myopathies, may occur. Management: If concomitant therapy is required, use the lowest dose for the shortest duration to limit the risk of myopathy or neuropathy. Monitor for new onset or worsening muscle weakness, reduction or loss of deep tendon reflexes, and peripheral sensory decrements *Risk D: Consider Therapy Modification*

CycloSPORINE (Systemic): May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Esketamine (Injection): May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Fosphenytoin-Phenytoin: May decrease neuromuscular-blocking effects of Neuromuscular-Blocking Agents (Nondepolarizing). Specifically, this effect is observed with chronic phenytoin administration. Fosphenytoin-Phenytoin may increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents (Nondepolarizing). Specifically, this effects is observed with acute/short term phenytoin administration. *Risk C: Monitor*

Gepotidacin: May decrease therapeutic effects of Neuromuscular-Blocking Agents (Nondepolarizing). *Risk C: Monitor*

Inhalational Anesthetics: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents (Nondepolarizing). Management: When initiating a non-depolarizing neuromuscular blocking agent (NMBA) in a patient receiving an inhalational anesthetic, initial NMBA doses should be reduced 15% to 25% and doses of continuous infusions should be reduced 30% to 60%. *Risk D: Consider Therapy Modification*

Ketamine: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Ketorolac (Nasal): May increase adverse/toxic effects of Neuromuscular-Blocking Agents (Nondepolarizing). Specifically, episodes of apnea have been reported in patients using this combination. *Risk C: Monitor*

Ketorolac (Systemic): May increase adverse/toxic effects of Neuromuscular-Blocking Agents (Nondepolarizing). Specifically, episodes of apnea have been reported in patients using this combination. *Risk C: Monitor*

Lincosamide Antibiotics: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Lithium: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Local Anesthetics: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Loop Diuretics: May decrease neuromuscular-blocking effects of Neuromuscular-Blocking Agents. Loop Diuretics may increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Magnesium Salts: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Minocycline (Systemic): May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Pancuronium: May increase adverse/toxic effects of Neuromuscular-Blocking Agents. Pancuronium may increase therapeutic effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Pholcodine: May increase adverse/toxic effects of Neuromuscular-Blocking Agents. Specifically, anaphylaxis has been reported. *Risk C: Monitor*

Polymyxin B: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Procainamide: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Quinidine: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Quinine: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk X: Avoid*

Tetracyclines: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Thiazide and Thiazide-Like Diuretics: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents (Nondepolarizing). *Risk C: Monitor*

Vancomycin: May increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Pregnancy Considerations

Small amounts of atracurium have been shown to cross the placenta when given to women during cesarean section.

Breastfeeding Considerations

It is not known if atracurium is present in breast milk. The manufacturer recommends that caution be exercised when administering atracurium to breastfeeding women.

Monitoring Parameters

Vital signs (heart rate, blood pressure, respiratory rate); degree of muscle paralysis (eg, presence of spontaneous movement, ventilator asynchrony, shivering, and use a peripheral nerve stimulator with train of four monitoring along with clinical assessments).

In the ICU setting, limiting the duration of paralysis and providing a physiotherapy regimen in patients requiring continuous paralysis is suggested.

Mechanism of Action

Blocks neural transmission at the myoneural junction by binding with cholinergic receptor sites

Pharmacokinetics (Adult Data Unless Noted)

Onset of action (dose dependent): 2 to 3 minutes; Peak effect: 3 to 5 minutes.

Duration: Recovery begins in 20 to 35 minutes following initial dose of 0.4 to 0.5 mg/kg under balanced anesthesia; recovery to 95% of control takes 60 to 70 minutes; hypothermia may prolong the duration of action.

Distribution (Fisher 1990): V_d :

Infants: 0.21 ± 0.12 L/kg.

Children: 0.13 ± 0.04 L/kg.

Adults: 0.1 ± 0.02 L/kg.

Metabolism: Undergoes ester hydrolysis and Hofmann elimination (nonbiologic process independent of kidney, hepatic, or enzymatic function); metabolites have no neuromuscular blocking properties; laudanosine, a product of Hofmann elimination, is a CNS stimulant and can accumulate with prolonged use. Laudanosine is hepatically metabolized.

Half-life elimination:

Infants: 20 ± 5.1 minutes (Fisher 1990).

Children: 17.2 ± 5.1 minutes (Fisher 1990).

Adults: Biphasic: Initial (distribution): 2 minutes; Terminal: 20 minutes.

Excretion: Urine (<5%).

Clearance (Fisher 1990):

Infants: 7.9 ± 0.2 mL/kg/minute.

Children: 6.8 ± 1.6 mL/kg/minute.

Adults: 5.3 ± 0.9 mL/kg/minute.

Patient Counseling Points

(For additional information see "Atracurium: Patient drug information")

What is this drug used for?

- It is used to relax muscles while on a breathing machine or during surgery.

All drugs may cause side effects. However, many people have no side effects or only have minor side effects. Call your doctor or get medical help if any of these side effects or any other side effects bother you or do not go away:

- Flushing

WARNING/CAUTION: Even though it may be rare, some people may have very bad and sometimes deadly side effects when taking a drug. Tell your doctor or get medical help right away if you have any of the following signs or symptoms that may be related to a very bad side effect:

- Very bad dizziness or passing out
- Fast or slow heartbeat
- Signs of an allergic reaction, like rash; hives; itching; red, swollen, blistered, or peeling skin with or without fever; wheezing; tightness in the chest or throat; trouble breathing, swallowing, or talking; unusual hoarseness; or swelling of the mouth, face, lips, tongue, or throat.

Note: This is not a comprehensive list of all side effects. Talk to your doctor if you have questions.

Consumer Information Use and Disclaimer: This information should not be used to decide whether or not to take this medicine or any other medicine. Only the healthcare provider has the knowledge and training to decide which medicines are right for a specific patient. This information does not endorse any medicine as safe, effective, or approved for treating any patient or health

condition. This is only a limited summary of general information about the medicine's uses from the patient education leaflet and is not intended to be comprehensive. This limited summary does NOT include all information available about the possible uses, directions, warnings, precautions, interactions, adverse effects, or risks that may apply to this medicine. This information is not intended to provide medical advice, diagnosis or treatment and does not replace information you receive from the healthcare provider. For a more detailed summary of information about the risks and benefits of using this medicine, please speak with your healthcare provider and review the entire patient education leaflet.

Brand Names: International

International Brand Names by Country

For country code abbreviations (show table)

- (AR) Argentina: Atracurio gray | Atracurium fabra | Atracurium northia | Atracurium pharmavial | Gelolagar | Tracuron;
- (AT) Austria: Atracurium hameln | Atracuriumbes deltaselect | Tracrium;
- (AU) Australia: Atracurium;
- (BD) Bangladesh: Atrelax | Tracrium;
- (BE) Belgium: Atracurium kalceks | Tracrium;
- (BG) Bulgaria: Atracurium kalceks;
- (BR) Brazil: Besilato de atracurio | Besilato de atracurio basi | Tracrium | Tracur;
- (CH) Switzerland: Atracurium Labatec | Tracrium;
- (CL) Chile: Besilato de atracurio;
- (CN) China: Atracurium;
- (CO) Colombia: Tracrium;
- (CR) Costa Rica: Besilato de atracurio;
- (CZ) Czech Republic: Atracurium kalceks;
- (DE) Germany: Atracurium | Atracurium Actavis | Atracurium hikma | Tracrium;
- (DO) Dominican Republic: Atracurio libra | Tracur;
- (EC) Ecuador: Atracur | Besilato de atracurio | Tracrium;
- (EE) Estonia: Atracurium | Atrasyl | Tracrium;
- (EG) Egypt: Atrabesylate | Atracurium | Faulcurium | Tracrium | Trathesia;
- (ES) Spain: Atracurio inibsa | Atracurio rovi | Besilato de atracurio hameln | Besilato de Atracurio Inibsa | Tracrium;
- (FI) Finland: Atracurium alpharm | Tracrium | Tracrium;
- (FR) France: Atracurium hikma | Tracrium;
- (GB) United Kingdom: Atracurium | Tracrium;
- (GR) Greece: Tracrium;
- (HU) Hungary: Atracurium besilate kalceks | Tracrium;
- (ID) Indonesia: Atracurium | Atracurium besylat | Farelax | Notrixum | Tracrium;
- (IE) Ireland: Atracurium | Tracrium;
- (IL) Israel: Tracrium;
- (IN) India: Acris | Arium | Artacil | Atacurium | Atcuron | Atraaish k | Atracade | Atrager | Crimat | Kabitran | Tracrium | Troycurium;
- (IQ) Iraq: Atracural;
- (IT) Italy: Acurmil | Tracrium;
- (JO) Jordan: Atacure | Retalex | Tracrium;
- (KE) Kenya: Acuron | Artacil | Atracurium | Atracurium besylate;
- (KR) Korea, Republic of: Acrum | Aculex | Atra | Tracrium;
- (KW) Kuwait: Tracrium;
- (LB) Lebanon: Tracrium;
- (LT) Lithuania: Atracurium hikma | Tracrium;
- (LU) Luxembourg: Tracrium;
- (LV) Latvia: Atracurium | Atracurium besilate kalceks | Tracrium;

- (MA) Morocco: Tracrium;
- (MX) Mexico: Atracurio Senosiain | Relatrac | Tracrium;
- (MY) Malaysia: Atracurium | Atracurium hameln | Atracurium kalceks | Atralex | Notrixum;
- (NL) Netherlands: Atracurium | Atracuriumdibesilaat | Tracrium;
- (NO) Norway: Atracurium | Tracrium | Tracrium aspen;
- (NZ) New Zealand: Atracurium besylat;
- (PA) Panama: Besilato de atracurio;
- (PE) Peru: Atracurio;
- (PH) Philippines: Atracor | Atracurium besylate | Atracurium Hospira | Atravell | Atrium | Tracrium;
- (PK) Pakistan: Acuron | Atrelax | Efacurim | Relaxtron | Tracrium | Tracur;
- (PL) Poland: Tracrium;
- (PR) Puerto Rico: Atracurium besylate | Tracrium;
- (PT) Portugal: Besilato de atracurio basi | Besilato de atracurio hikma | Besilato de atracurio queenlabs | Faulcurium | Tracrium;
- (PY) Paraguay: Arium | Atracurio libra | Atracurium northia | Tracur | Tracurix;
- (QA) Qatar: Neucurium | Tracrium;
- (RO) Romania: Atracurium kalceks;
- (RU) Russian Federation: Atracuria besylate | Atracurium belmed | Atracurium besilat | Atracurium medargo | Atracurium novo | Notrixum | Ridelat c | Tracrium;
- (SA) Saudi Arabia: Atacure | Atracurium besylate | Retalex;
- (SE) Sweden: Tracrium;
- (SG) Singapore: Atracurium | Atracurium hameln;
- (SI) Slovenia: Atracurium besilate kalceks | Tracrium;
- (SK) Slovakia: Atracurium kalceks;
- (TH) Thailand: Notrixum;
- (TN) Tunisia: Atracurium;
- (TR) Turkey: Atracurium | Atrasyl | Dematrac | Neucurium;
- (TW) Taiwan: Atracurium | Atracurium besylat | Atracurium besylate | Genso | Tracrium;
- (UA) Ukraine: Intuban;
- (UG) Uganda: Acuron | Retalex;
- (UY) Uruguay: Atracur | Atracurio | Atracurio blaufarma uruguay | Atracurium | Tracrium | Tracur | Tracurix;
- (VE) Venezuela, Bolivarian Republic of: Tracrium;
- (ZA) South Africa: Atracurium | Tracrium;
- (ZW) Zimbabwe: Tracrium

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